

A vertical image on the left side of the slide showing the Memphis skyline at sunset. The sky is a mix of orange, pink, and purple. The city lights are visible, and the Mississippi River flows in the foreground with a bridge in the distance.

ShelbyFirst

Advancing Inclusive Growth in Low-IGS Shelby County Through Health Access, Nutrition, and Affordable Mobility

Community Analyzed: Shelby County / Memphis Low-Growth Recovery Cluster

Challenge Topic: Inclusive Growth / Community Health Access

96

Low-IGS Tracts

273K

Population

35.8

Avg IGS Score

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

Data-driven selection process

Why this community, and why this problem

1

Economic Vulnerability

- IGS Economy
- poverty
- unemployment
- median household income
- labor-force participation

2

Compared communities across three periods

- pre-COVID: 2017-2019
- COVID: 2020-2021
- recovery: 2022-2024

3

Scored communities based on

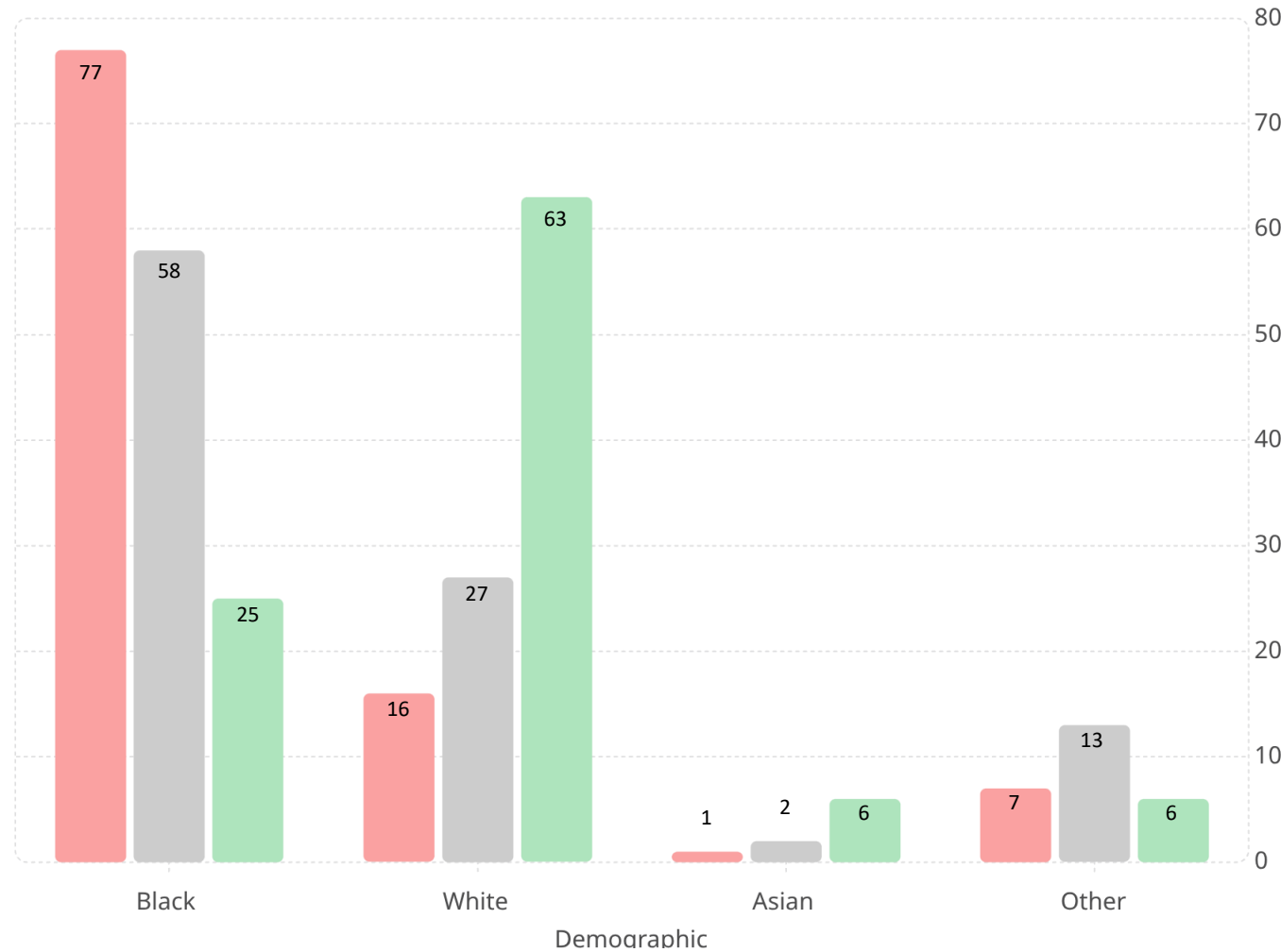
- low recovery-period IGS
- economic vulnerability in recovery
- COVID shock
- weak recovery

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

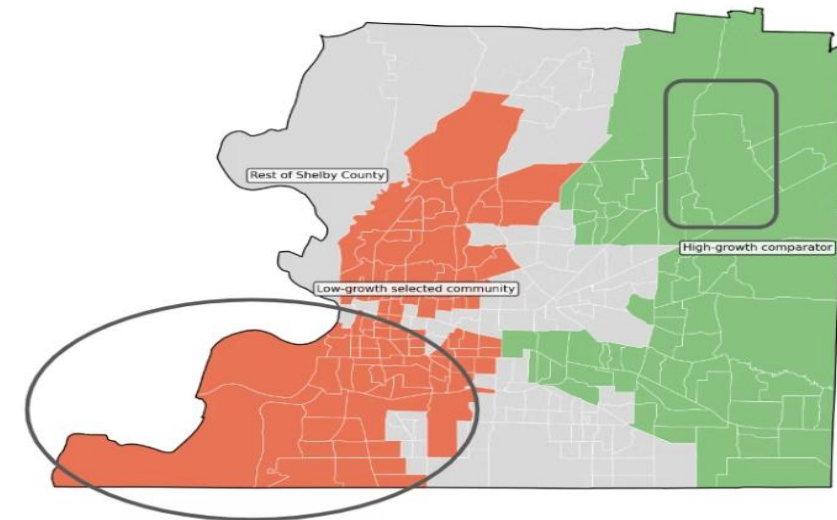
Community Overview

A large, clearly vulnerable Shelby recovery cluster

Our Shelby Community Rest of Shelby County High-Growth Shelby Community



Shelby County: low-growth selected community vs high-growth comparator



Whitehaven ZIPs of Focus

ZIP	Population	Area
38109	~50K	62 sq mi
38116	~40K	12 sq mi
38117	~26K	9 sq mi

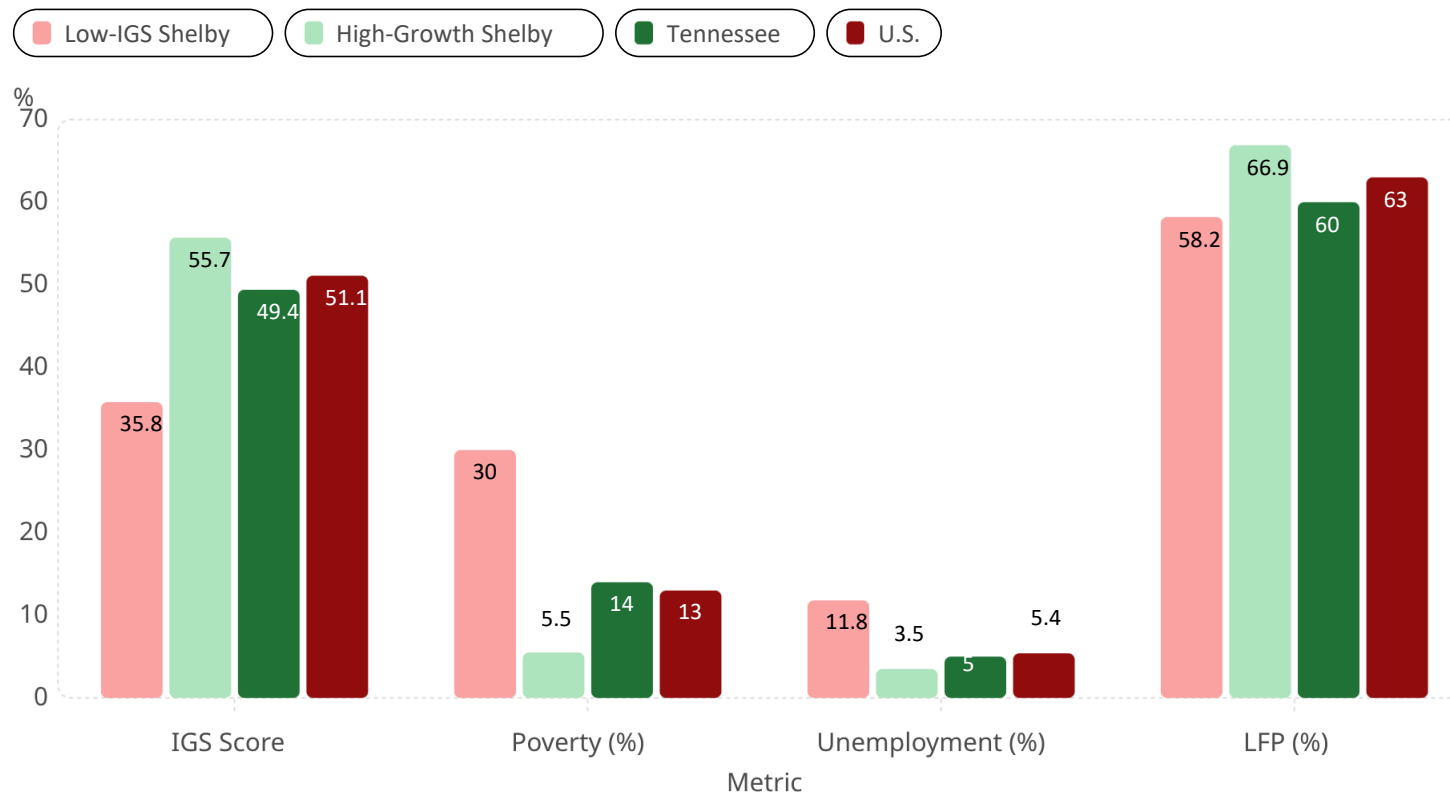
❏ **Economy, mobility, food, and healthcare access intersect here.** This is a concentrated, addressable geography with overlapping vulnerabilities.

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

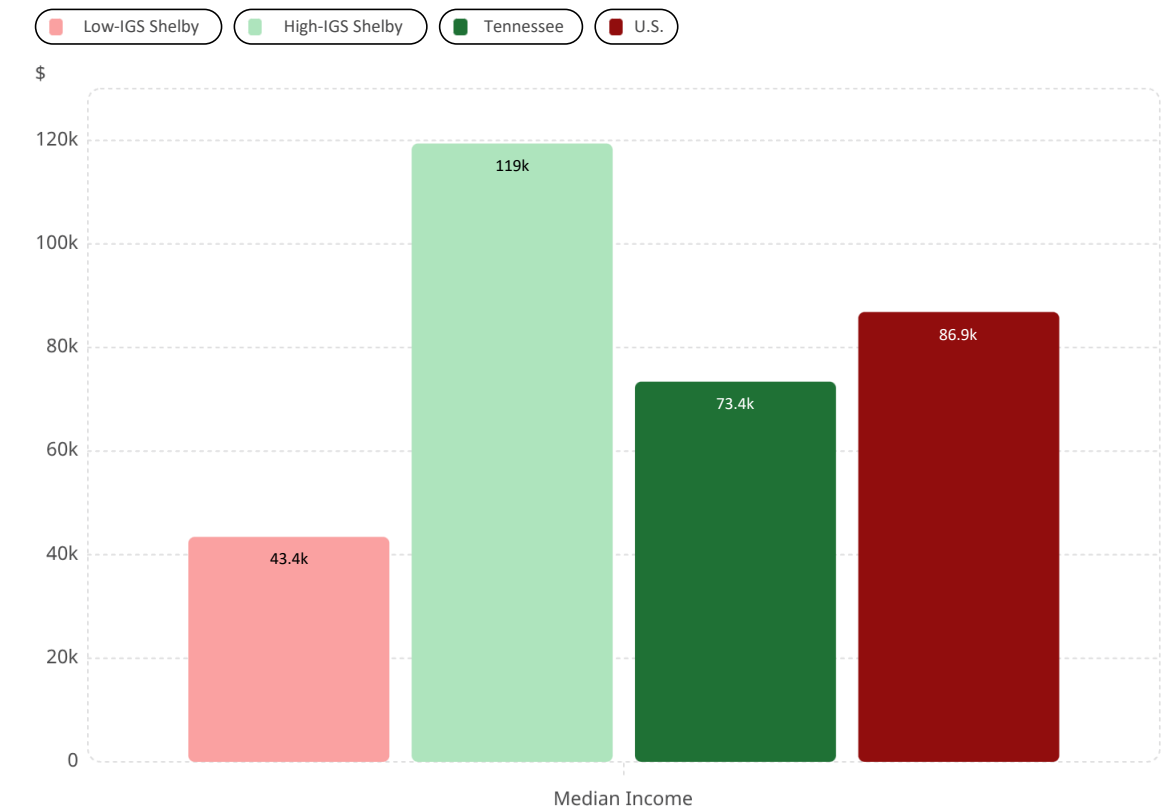
Benchmarking

This community underperforms county, state, and national benchmarks

Economic Gaps



Median Income

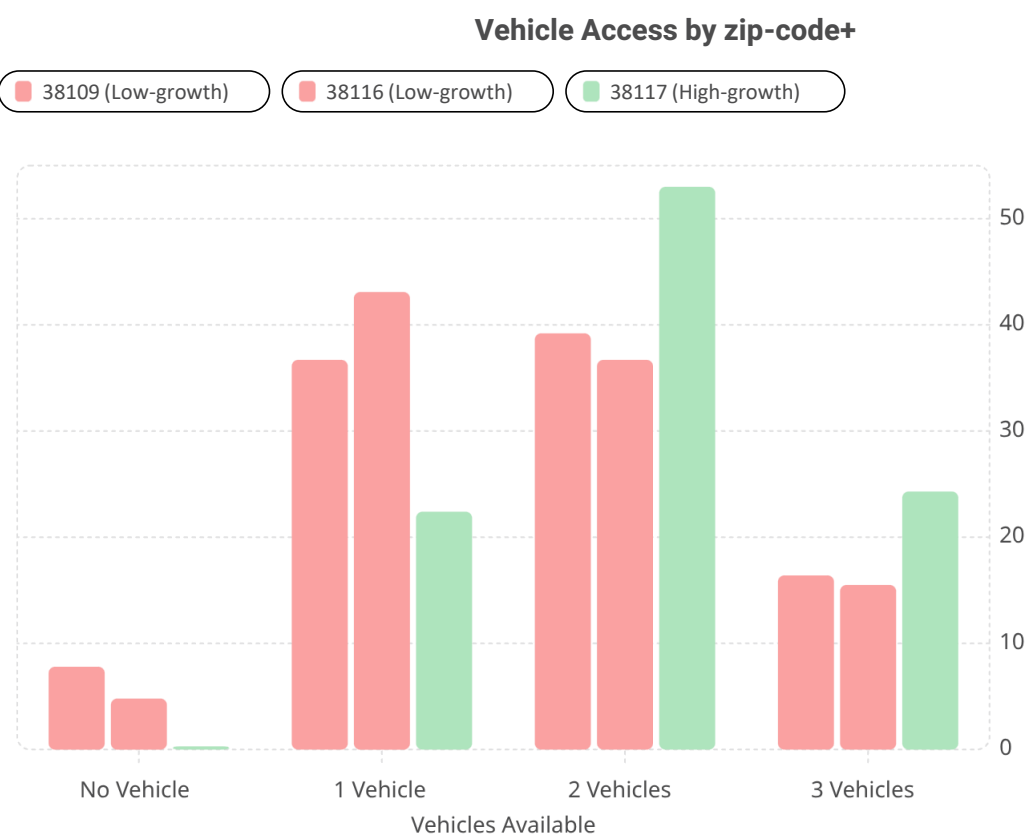


📌 "Low-IGS Shelby is not slightly behind – it is structurally disconnected from the county's higher-growth trajectory."

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

Key Findings

Economic vulnerability overlaps with worse healthcare access and health strain



1. Compounding Health-Access Penalty

Low-IGS tracts face a compounding health-access penalty across insurance, providers, and preventive care.

2. Transportation Friction

Transportation friction appears to reduce preventive and dental care completion rates.

The strongest story: Economic vulnerability + transportation/access friction → lower preventive care + missed appointments → worse chronic disease burden.

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

Implementation Plan

A practical 3-year rollout built on existing Memphis health infrastructure



Phase 1 — Pilot (Year 1)

- Launch in highest-need South Memphis service area
- Start one community garden site
- Build referral and ride coordination with ShelbyCares and trusted providers
- Enroll founding households and begin subsidized health rides



Phase 2 — Expand (Year 2)

- Add more ride destinations and health event partners
- Expand produce distribution and awareness
- Grow membership base and app usage



Phase 3 — Scale (Year 3)

- Expand across more low-IGS tracts
- Formalize healthcare referral loops
- Move toward blended nonprofit sustainability

Funding Architecture

Startup & Pilot Grants

Public-health and foundation grant funding for Year 1 launch

Institutional Partnerships

ShelbyCares, UTHSC-aligned care ecosystem, local government, and public-health stakeholders

Blended Earned Revenue

Memberships, subsidized rides, and produce/garden programs

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

Proposed Solutions

ShelbyFirst connects mobility, nutrition, and care access in one platform



Move

Affordable healthcare-focused rides priced below typical rideshare alternatives.
Member: \$4–\$6 one-way / ~\$8 round trip in-zone
Non-member: \$7–\$9 one-way



Nourish

Community garden in southern Memphis providing healthy food access, physical activity, and community participation.
Optional add-on: \$10–\$15/month



Connect

App integrating ride access, garden engagement, health events, referrals, and partner services including ShelbyCares.
Access Membership: \$9.99/mo | **Care Membership:** \$19.99/mo (2 subsidized rides)



Google Docs

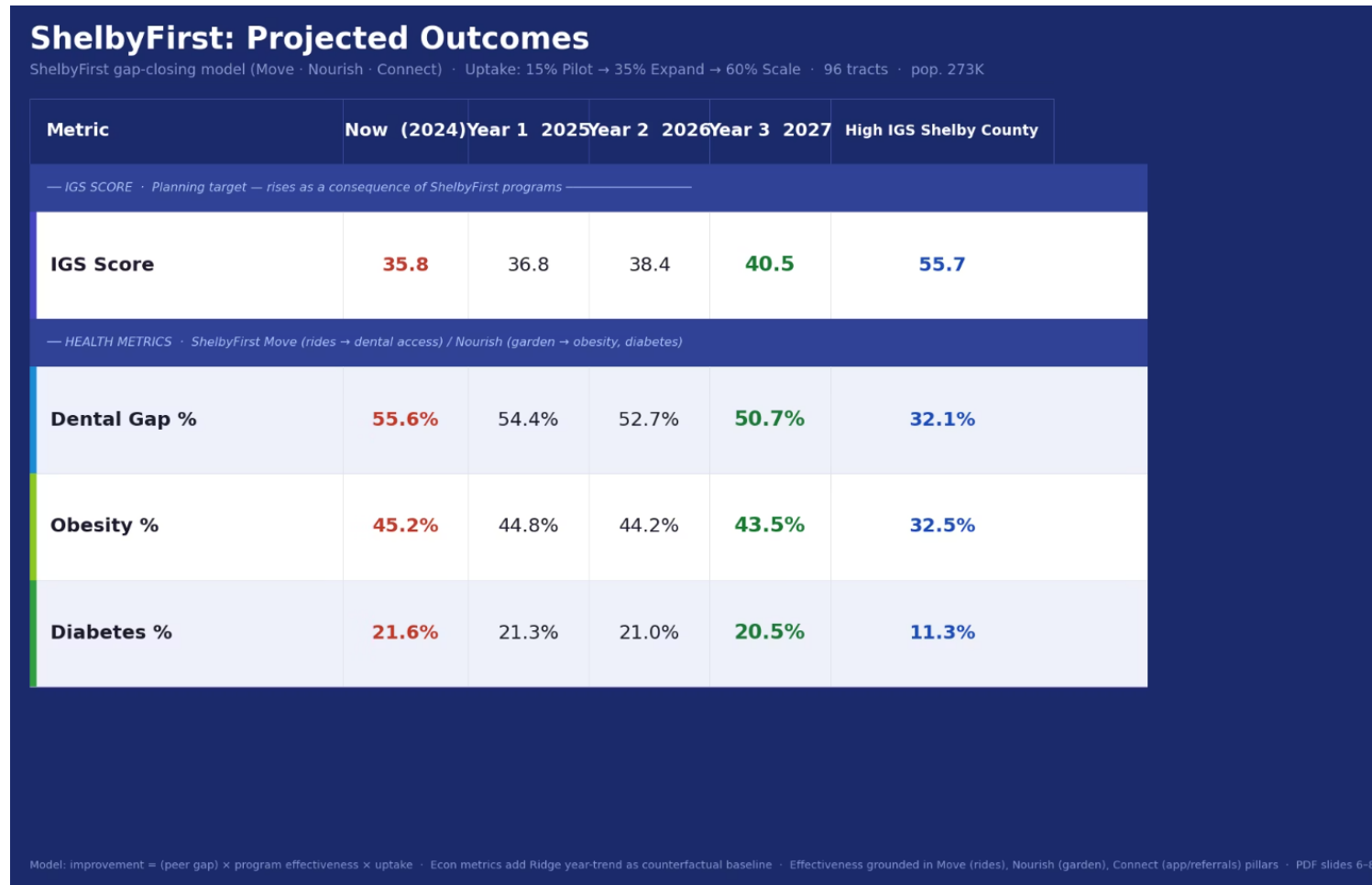
Screen_Recording_20260317_010201_Chrome.mp4



Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

Predicted Outcomes

A realistic 3-year pathway to healthier and more inclusive growth



Base-Case Story

- 1** Year 1 — Modest Pilot Effects
Founding household enrollment, initial ride utilization, garden launch, early referral loops with ShelbyCares.
- 2** Year 2 — Stronger Engagement
Growing membership, dental screening events, expanded produce distribution, preventive-care campaign uptake.
- 3** Year 3 — Measurable Improvements
Measurable improvements in utilization and access; directional uplift in Inclusive Growth performance over time.

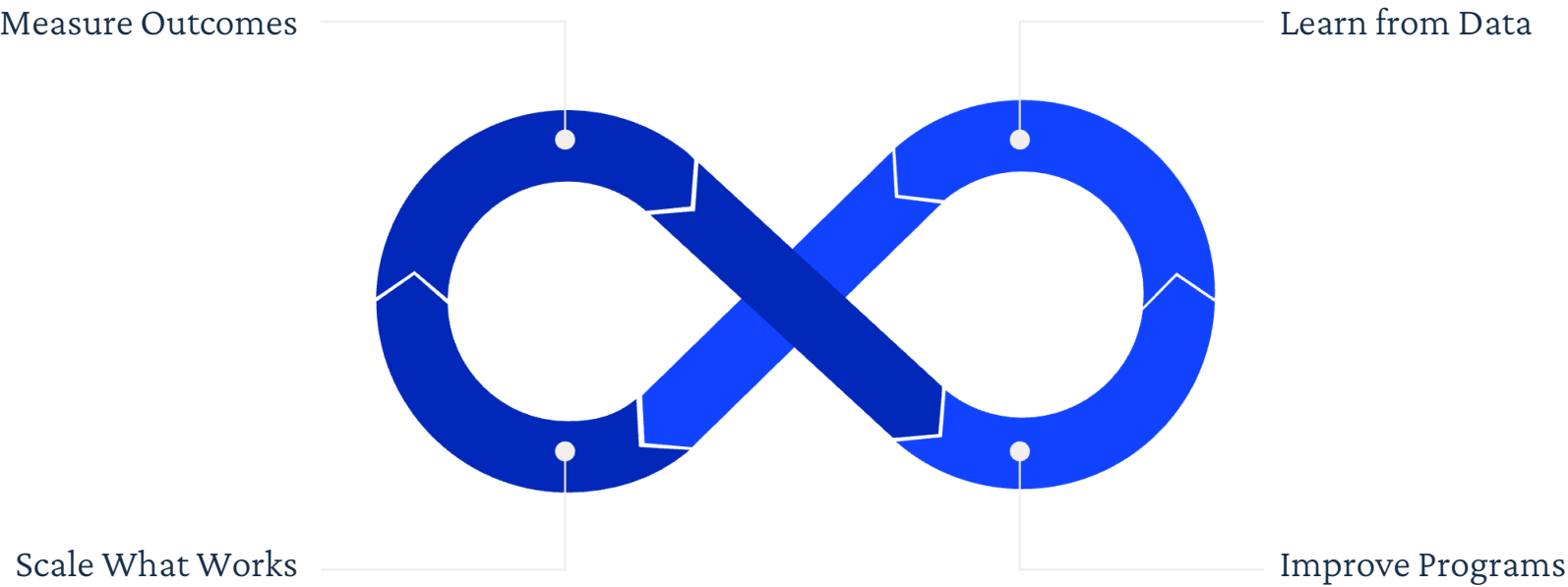
📌 **ShelbyFirst influences IGS indirectly** through better mobility, healthier food access, stronger preventive care participation, and deeper community engagement.

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

Metrics for Success

How ShelbyFirst will track impact and accountability over time

			
Access • Active members • Rides completed • Average ride cost • % of rides to healthcare destinations	Care Utilization • Appointment attendance • Dental/preventive visit completion • Referral-to-appointment conversion • Repeat clinic engagement	Nutrition & Community • Garden participation • Produce distributed • Class/workshop attendance • Repeat engagement	Inclusive Growth Monitoring • Ongoing comparison of target tracts against IGS-linked indicators over time • Transportation access proxies • Preventive care proxies • Community-level health access measures



Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.

Conclusion

ShelbyFirst transforms disconnected resources into a pathway for healthier, more inclusive growth



Economically Vulnerable & Health-Access Constrained

The selected Shelby community faces overlapping disadvantage across income, transportation, preventive care, diabetes, obesity, and dental access.



Practical, Community-Based Intervention

ShelbyFirst coordinates affordable rides, healthy food access, and trusted care connectivity — using existing infrastructure, not building new systems.



Built for Measurable Impact

The initiative is built for measurable impact, not just awareness — with clear metrics, phased rollout, and blended nonprofit sustainability.

"ShelbyFirst turns mobility, nutrition, and care access into a scalable engine for inclusive growth in Shelby County."

Healthier Residents

Stronger Communities

More Inclusive Growth

Source: IGS, ACS/Census, healthcare enrichment, uploaded analysis charts.